NEELEY AI FORWARD: AI INNOVATION COMPETITION
ADVANCED TRACK CASE STUDY

The Fine Print: Find the Story, Then Make the Call

Dig into a pharmaceutical company's own disclosures and its competitors' fine print to find what the headlines miss. Then take the CEO's seat and commit to one strategic move. Judges reward careful reading and evidence that holds up. A stack of AI output earns nothing on its own.

Background

Every major drugmaker faces the same squeeze: blockbusters lose exclusivity, and Washington is rewriting how drugs get priced. Wall Street reads the headline numbers: revenue up, margins steady, pipeline "robust." The story worth telling is usually buried deeper, in a risk factor that got longer, a footnote that changed, or a question an executive dodged on an earnings call.

The case has two parts.

Part 1: Find, you uncover findings about one company that are material (big enough to change an investor's view) and not obvious, and you compare one of its key products against its rivals.

Part 2: Decide, you use those findings to recommend one strategic move as if you were running the company, and show why it would work.

How to choose your company

Each team picks one company from the shortlist below and should be ready to explain why it beat the other nine. Each company comes with a well-known "starter thread," the story everyone already knows. Restating it will not score well; the goal is to find what it misses.

Each company also has a competitive set. In Part 1 you will choose **one product** from that company and compare it with the rivals listed for that product. You don't need to cover rivals listed for the company's other products, and you may add competitors that aren't listed if you can justify them.

Sample companies and competitive sets are listed in the appendix of the case document.

What is happening today

Three forces hit all ten companies, just not equally. The differences are where you should dig:

- **Medicare drug-price negotiation** under the Inflation Reduction Act (IRA), which sets government-negotiated prices for some of the best-selling drugs. New drugs are added each year.
- **Most Favored Nation (MFN) pricing deals:** agreements with the U.S. government that cut some U.S. prices toward what other wealthy countries pay, in exchange for commitments such as U.S. manufacturing investment. All ten shortlist companies have signed one.
- **Tariffs on imported patented drugs:** a 100% U.S. tariff took effect in 2026, but companies with MFN deals are exempt until January 2029. What each company promised in return, and what happens after 2029, varies.

Key terms

- **Exclusivity:** the period when no cheaper copy can be sold. It can end before or after the main patent does.
- **Generic / biosimilar:** a cheaper copy of a drug. A biosimilar is the near-copy of a biologic (a drug made from living cells).

- **Priced in:** already reflected in the stock price because investors know about it.
- **10-K, 10-Q, 20-F:** annual (10-K) and quarterly (10-Q) reports U.S. companies file with the SEC. Foreign companies such as Novo Nordisk and AstraZeneca file an annual 20-F instead.
- **Gross-to-net:** the gap between list price and what the company actually collects after rebates and discounts.
- **Payers:** insurers, pharmacy benefit managers, and government programs like Medicare that decide what gets covered.
- **Boxed warning:** the FDA's most serious safety warning on a drug label.
- **Head-to-head evidence:** a study that directly compares two drugs.

Source material

Load the actual documents into your AI tools. A model working from memory will get numbers and quotes wrong. Use:

- The two most recent annual reports (10-K or 20-F), for year-over-year comparison
- The last four quarterly earnings call transcripts
- The most recent quarterly report (10-Q or equivalent)
- For each rival product you compare against: what its maker says about it in that company's latest annual report and earnings call
- The current FDA label for your product and each rival: approved uses, boxed warnings, who should not take it, and dosing
- Patent and exclusivity dates, which are not on the label: use the FDA's Orange Book (most drugs) or Purple Book (biologics), and the patent table in each company's 10-K

Filings are free on the SEC's EDGAR site and company investor-relations pages. Labels are on DailyMed or Drugs@FDA. Use any AI tools you like.

Where findings hide

- **Language drift:** wording about risks, results, or guidance that changed between years. Look especially at Risk Factors and at MD&A, the section where management discusses results.
- **Gross-to-net:** a widening gap between list price and net price
- **Inventory and receivables growing faster than revenue:** a possible sign of softening demand, pre-launch stocking, or channel stuffing (shipping more to distributors than patients are buying)
- **Segment and product table changes:** past numbers revised or reclassified, products quietly no longer broken out
- **Footnotes:** litigation reserves, partner royalty splits, payments owed on past acquisitions, tax-rate changes
- **Earnings-call Q&A:** deflected questions, new phrases, metrics the company stopped reporting
- **Product fine print:** label details that widen or narrow a product's market compared with its rivals, such as the number of approved uses, boxed warnings, and required safety programs
- **Peer contrast:** something the company or product does differently (pricing, geography, manufacturing footprint, label breadth, exposure to the three forces above) that the market hasn't priced in

Why this is a challenge

- Patent cliffs and exclusivity losses are public and priced in. The points go to signals a press-release skim would miss.
- Every finding needs a citation and an estimate of how much it matters. AI tools invent page numbers, mix up figures from different years, and misquote executives. Check everything against the source.
- Rival products are sold by different companies, on different reporting calendars, under different labels. Lining them up fairly takes analysis; you won't find the comparison in one place.
- Part 2 has no answer key. AI will argue convincingly for almost any strategy. Your team has to pick one and defend it.
- You have one weekend. You can't go deep on everything, so choose.

Minimum expectations

Part 1: Find

Financial forensics

- Pick one company and explain why it offered better material than the other nine.
- Deliver at least five findings in a table, one row per finding, with:
  - **Finding:** one plain-English sentence
  - **Source:** filing and page number, or for earnings calls, the speaker's name and exact quote
  - **Impact:** estimated effect on revenue, operating income, or earnings per share, with the math shown
  - **Peer comparison:** how the same item looks at another shortlist company or a rival product's maker
  - **Confidence:** high, medium, or low, based on how solid the evidence is
- Name your "Sharpest Needle": the single most material and least-discussed finding.

Product fine print

- Choose one product from your company's competitive set and compare it with the rivals listed for it.
- Build a side-by-side comparison covering approved uses, boxed warnings and safety fine print, dosing and convenience, patent and exclusivity timeline, and pricing and gross-to-net exposure.
- Tie the comparison to at least one financial finding: show where a label difference appears, or should appear, in the numbers. A product finding can count toward your five.
- State in one sentence what actually limits the product's growth, and whether the market has already priced it in.

Part 2: Decide

Take the CEO's seat. Based on your findings, recommend **one strategic move** for the next 24 months, with a specific budget or commitment of people behind it. Examples: acquiring or licensing a drug, changing a product's pricing or insurance coverage strategy, shifting investment across the portfolio, seeking approval for a new use, or launching a marketing campaign. Other moves are welcome. Include:

- **The move,** in one sentence.
- **The case for it:** which Part 1 findings drive the decision, what it costs, and what it should return, with the math shown.

- **Alternatives you rejected:** the two strongest options you considered and why you passed.
- **What would prove you wrong:** the early signals, in the first 6 to 12 months, that would tell leadership the move is failing.
- **Why it will work:** evidence that the move would land with the people who decide its fate, such as doctors, patients, payers, regulators, or investors. How you gather it is up to you. Whatever you use, explain what it can and cannot tell you.

Rules for anything a doctor or patient would see. If your move includes a campaign, messaging, or other materials, follow the rules for prescription drug promotion:

- Make claims only for FDA-approved uses.
- Show risks as prominently as benefits.
- Compare with a competitor only where head-to-head evidence supports it.
- Don't invent testimonials or use real people's likenesses.
- Label every piece "Student concept, not actual promotional material."
- Include a claims map: a table listing each factual claim next to the label section or study that supports it.

Final deliverables

- **Pitch deck with appendix.** Cover your company choice, the findings table, your Sharpest Needle, the product comparison, and your Part 2 recommendation. Put supporting math, the claims map, and any campaign materials in the appendix.
- **A reusable AI build:** a skill file, custom GPT or Claude project, no-code automation, AI-driven spreadsheet, or script that you used in your analysis and that could be pointed at a different company and still produce useful output. Submit the build itself or a short recording of it running. A one-off chat does not count.
- **How we did it:** a full report on how your team did the analysis. Format is up to you. Include:
  - The tools you used and what each was for
  - Your workflow: how documents got into the AI, how work was split between people and tools, and how outputs were checked
  - Your most important prompts
  - At least one case where the AI was confidently wrong, and how you caught it
  - What you would do differently

Submissions are judged against the competition's standard rubric.

This case sets a floor. Where did the filings raise a question we didn't ask? What would you chase with another week? The teams that win usually find an angle we didn't see coming.

Appendix: Company Shortlist & Competitive Sets

- **Merck:** Keytruda's 2028 patent expiry; Januvia and Janumet lost U.S. exclusivity in 2026 and now face generics and a Medicare negotiated price.
  - **Competitive set:** For Keytruda: Bristol Myers Squibb's Opdivo, AstraZeneca's Imfinzi, and Roche's Tecentriq (rival immunotherapies). For Januvia: generic sitagliptin, plus Novo Nordisk's Ozempic and Boehringer Ingelheim/Lilly's Jardiance (newer diabetes drugs that took share).

- **Bristol Myers Squibb:** Eliquis and Opdivo both lose U.S. exclusivity in 2028, putting a large share of revenue at risk.
  - **Competitive set:** For Eliquis: Johnson & Johnson's Xarelto (rival blood thinner). For Opdivo: Merck's Keytruda and Roche's Tecentriq (rival immunotherapies).
- **Pfizer:** Ibrance faces generic pressure from 2027 while the company rebuilds its growth story.
  - **Competitive set:** For Ibrance: Eli Lilly's Verzenio and Novartis's Kisqali (rival breast cancer drugs). For the Vyndaqel family: BridgeBio's Attruby and Alnylam's Amvuttra (rival heart disease drugs).
- **Eli Lilly:** Mounjaro and Zepbound lead the obesity market, but price cuts and the race for a pill are reshaping it; Trulicity loses exclusivity in 2027.
  - **Competitive set:** For Mounjaro/Zepbound and Trulicity: Novo Nordisk's Ozempic and Wegovy injections and the Wegovy pill.
- **Novo Nordisk:** Medicare's negotiated price for semaglutide (the active ingredient in Ozempic and Wegovy) takes effect January 2027, about 71% below the 2025 list price.
  - **Competitive set:** For Ozempic/Wegovy: Eli Lilly's Mounjaro and Zepbound injections and its Foundayo pill.
- **AbbVie:** Life after Humira: how fast Skyrizi and Rinvoq are filling the revenue gap.
  - **Competitive set:** For Skyrizi/Rinvoq: Johnson & Johnson's Tremfya and Stelara, Novartis's Cosentyx, and Takeda's Entyvio (rival immunology drugs), plus cheaper Humira biosimilars such as Amgen's Amjevita.
- **Johnson & Johnson:** Stelara biosimilar erosion, Medicare drug-price negotiation, and the shift toward newer immunology and cancer drugs.
  - **Competitive set:** For Stelara/Tremfya: AbbVie's Skyrizi and Rinvoq, and Amgen's Wezlana (a Stelara biosimilar). For Darzalex: Sanofi's Sarclisa (rival multiple myeloma drug).
- **AstraZeneca:** Cancer-drug growth, near-term exclusivity losses (generic Farxiga was approved in 2026), and exposure to U.S. pricing politics as a UK-based company.
  - **Competitive set:** For Tagrisso: Johnson & Johnson's Rybrevant plus Lazcluze (rival lung cancer treatment). For Farxiga: generic dapagliflozin and Boehringer Ingelheim/Lilly's Jardiance. For Imfinzi: Merck's Keytruda.
- **Amgen:** Enbrel and Repatha lose U.S. exclusivity around 2029 while Amgen bets on obesity drugs.
  - **Competitive set:** For Repatha: Regeneron's Praluent, Novartis's Leqvio, and Merck's Lipfendra (the first pill in this class). For Enbrel: AbbVie's Skyrizi and Rinvoq and Humira biosimilars. Background only: Amgen's obesity drug MariTide is still in trials, so it can't be your Part 1 product, but Eli Lilly's Zepbound and Novo Nordisk's Wegovy are the incumbents it would have to beat.
- **Gilead:** A dominant HIV franchise, and how long that dominance can hold.
  - **Competitive set:** For Biktarvy: ViiV Healthcare's Dovato and Cabenuva. For Yeztugo (HIV prevention): ViiV's Apretude. ViiV is owned by GSK and Shionogi.